

Pharmacy Policy Bulletin: J-0561 Chronic Inflammatory Diseases - Medicare	
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Line(s) of Business: ☐ Commercial ☐ Healthcare Reform ☒ Medicare	Benefit(s): Not applicable
Region(s): ☒ All ☐ Delaware ☐ New York ☐ Pennsylvania ☐ West Virginia	Additional Restriction(s): None
Version: J-0561-060	Original Date: 08/10/2017
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Drugs Product(s):	• Abrilada (adalimumab-afzb) subcutaneous (SC) injection • Actemra (tocilizumab) SC injection, intravenous (IV) infusion • Amjevita (adalimumab-atto) SC injection • Avsola (infliximab-axxq) IV infusion • Bimzelx (bimekizumab-bkzx) SC injection • Cimzia (certolizumab) SC injection • Cosentyx (secukinumab) SC injection, IV infusion • Cyltezo (adalimumab-adbm) SC injection • Enbrel (etanercept) SC injection • Entyvio (vedolizumab) SC injection, IV infusion • Hadlima (adalimumab-bwwd) SC injection • Hulio (adalimumab-fkjp) SC injection • Humira (adalimumab) SC injection • Hyrimoz (adalimumab-adaz) SC injection • Idacio (adalimumab-aacf) SC injection • Ilumya (tildrakizumab) SC injection • Infliximab IV infusion • Inflectra (infliximab-dyyb) IV infusion • Kevzara (sarilumab) SC injection • Kineret (anakinra) SC injection • Litfulo (ritlecitinib) oral capsule • Olumiant (baricitinib) oral tablet • Omvoh (mirikizumab-mrkz) SC injection, IV infusion • Orencia (abatacept) SC injection, IV infusion • Otezla (apremilast) oral tablet • Remicade (infliximab) IV infusion • Renflexis (infliximab-abda) IV infusion • Rinvoq (upadacitinib) oral tablet • Rinvoq LQ (upadacitinib) oral solution • Siliq (brodalumab) SC injection • Simlandi (adalimumab-ryvk) SC injection • Simponi (golimumab) SC injection • Simponi Aria (golimumab) IV infusion
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	• Skyrizi (risankizumab) SC injection, IV infusion • Sotyktu (deucravacitinib) oral tablet • Stelara (ustekinumab) SC injection, IV infusion • Taltz (ixekizumab) SC injection • Tofidence (tocilizumab-bavi) IV infusion • Tremfya (guselkumab) SC injection, IV infusion • Tyenne (tocilizumab-aazg) SC injection, IV infusion • Velsipity (etrasimod) oral tablet • Xeljanz, Xeljanz XR (tofacitinib) oral tablet, oral solution • Yuflyma (adalimumab-aaty) SC injection • Yusimry (adalimumab-aqvh) SC injection • Zeposia (ozanimod) oral capsule • Zymfentra (infliximab-dyyb) SC injection
FDA- Approved Indication(s):	• Actemra SC ○ Treatment of adult patients with moderately to severely active rheumatoid arthritis (RA) who have had an inadequate response to one or more disease-modifying anti-rheumatic drugs (DMARDs) ○ Treatment of adult patients with giant cell arteritis (GCA) ○ Slowing the rate of decline in pulmonary function in adult patients with systemic sclerosis-associated interstitial lung disease (SSc-ILD) ○ Treatment of polyarticular juvenile idiopathic arthritis (PJIA) in patients 2 years of age and older ○ Treatment of patients 2 years of age and older with active systemic juvenile idiopathic arthritis (SJIA) • Actemra IV ○ Treatment of adult patients with moderately to severely active rheumatoid arthritis (RA) who have had an inadequate response to one or more disease-modifying anti-rheumatic drugs (DMARDs) ○ Treatment of patients 2 years of age and older with active polyarticular juvenile idiopathic arthritis (PJIA) ○ Treatment of patients 2 years of age and older with active systemic juvenile idiopathic arthritis (SJIA) ○ Treatment of adults and pediatric patients 2 years of age and older with chimeric antigen receptor (CAR) T cell-induced severe or life-threatening cytokine release syndrome (CRS) ○ Treatment of hospitalized adult patients with coronavirus disease 2019 (COVID-19) who are receiving systemic corticosteroids and require supplemental oxygen, non-invasive or invasive mechanical ventilation, or extracorporeal membrane oxygenation (ECMO) • Adalimumab Biosimilars ○ Reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active rheumatoid arthritis (RA) ○ Reducing signs and symptoms of moderately to severely active polyarticular juvenile idiopathic arthritis (PJIA or JIA) in pediatric patients 2 years of age and older ○ Reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active psoriatic arthritis (PsA) ○ Reducing signs and symptoms in adult patients with active ankylosing spondylitis (AS) ○ Treatment of moderately to severely active Crohn's disease (CD) in adults and pediatric patients 6 years of age and older.

	○ Treatment of moderately to severely active ulcerative colitis (UC) in adult patients. ○ Treatment of adult patients with moderate to severe chronic plaque psoriasis (PsO) who are candidates for systemic therapy or phototherapy, and when other systemic therapies are medically less appropriate. ○ Treatment of moderate to severe hidradenitis suppurativa (HS) in adult patients. ○ Treatment of non-infectious intermediate, posterior, and panuveitis (UV) in adult patients. • Bimzelx ○ Treatment of moderate to severe plaque psoriasis (PsO) in adults who are candidates for systemic therapy or phototherapy • Cimzia ○ Treatment of adult patients with moderately to severely active rheumatoid arthritis (RA) ○ Reducing signs and symptoms in adult patients with active ankylosing spondylitis (AS) ○ Treatment of adult patients with active psoriatic arthritis (PsA) ○ Reducing signs and symptoms and maintaining clinical remission in adult patients with moderately to severely active Crohn's disease (CD) who have had an inadequate response to conventional therapy ○ Treatment of adult patients with moderate to severe chronic plaque psoriasis (PsO) who are candidates for systemic therapy or phototherapy ○ Treatment of adults with active non-radiographic axial spondyloarthritis (nr-axSpA) with objective signs of inflammation • Cosentyx SC ○ Treatment of adult patients with active ankylosing spondylitis (AS) ○ Treatment of active psoriatic arthritis (PsA) in patients 2 years of age and older ○ Treatment of moderate to severe plaque psoriasis (PsO) in patients 6 years and older who are candidates for systemic therapy or phototherapy ○ Treatment of adult patients with active non-radiographic axial spondyloarthritis (nr-axSpA) with objective signs of inflammation ○ Treatment of active enthesitis-related arthritis (ERA) in patients 4 years of age and older ○ Treatment of adults with moderate to severe hidradenitis suppurativa (HS) • Cosentyx IV ○ Treatment of adult patients with active ankylosing spondylitis (AS) ○ Treatment of adult patients with active psoriatic arthritis (PsA) ○ Treatment of adult patients with active non-radiographic axial spondyloarthritis (nr-axSpA) with objective signs of inflammation • Enbrel ○ Reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active rheumatoid arthritis (RA) ○ Reducing signs and symptoms in adult patients with active ankylosing spondylitis (AS) ○ Reducing signs and symptoms of moderately to severely active polyarticular juvenile idiopathic arthritis (PJIA or JIA) in pediatric patients 2 years of age and older
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- Reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active **psoriatic arthritis** (PsA)
- Treatment of patients 4 years or older with chronic moderate to severe **plaque psoriasis** (PsO) who are candidates for systemic or phototherapy
- Treatment of active **juvenile psoriatic arthritis** (JPsA) in pediatric patients 2 years of age and older
- **Entyvio**
 - Treatment of moderately to severely active **ulcerative colitis** (UC) in adults.
 - Treatment of moderately to severely active **Crohn's disease** (CD) in adults.
- **Humira**
 - Reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active **rheumatoid arthritis** (RA)
 - Reducing signs and symptoms in adult patients with active **ankylosing spondylitis** (AS)
 - Reducing signs and symptoms of moderately to severely active **polyarticular juvenile idiopathic arthritis** (PJIA or JIA) in pediatric patients 2 years of age and older
 - Reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active **psoriatic arthritis** (PsA)
 - Treatment of adult patients with moderate to severe chronic **plaque psoriasis** (PsO) who are candidates for systemic therapy or phototherapy, and when other systemic therapies are medically less appropriate
 - Reducing signs and symptoms and inducing and maintaining clinical remission in patients 6 years of age and older with moderately to severely active **Crohn's disease** (CD) who have had an inadequate response to corticosteroids or immunomodulators, such as azathioprine, 6-mercaptopurine, or methotrexate, or have lost response to or are intolerant to infliximab (Remicade)
 - Inducing and sustaining clinical remission in adult patients 5 years of age and older with moderately to severely active **ulcerative colitis** (UC) who have had an inadequate response to immunosuppressants such as corticosteroids, azathioprine or 6-mercaptopurine (6-MP)
 - Treatment of moderate to severe **hidradenitis suppurativa** (HS) in patients 12 years of age and older
 - Treatment of non-infectious **intermediate, posterior and panuveitis** (UV) in adults and pediatric patients 2 years of age and older
- **Ilumya**
 - Treatment of adult patients with moderate-to-severe **plaque psoriasis** (PsO) who are candidates for systemic therapy or phototherapy
- **Infliximab IV**
 - Reducing signs and symptoms in adult patients with active **ankylosing spondylitis** (AS)
 - Reducing signs and symptoms and inducing and maintaining clinical remission in patients 6 years of age and older with moderately to severely active **Crohn's disease** (CD) who have had an inadequate response to corticosteroids or immunomodulators such as azathioprine, 6-mercaptopurine, or methotrexate

- The treatment of adult patients with moderate to severe chronic **plaque psoriasis** (PsO) who are candidates for systemic therapy or phototherapy, and when other systemic therapies are medically less appropriate
- Reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active **psoriatic arthritis** (PsA)
- Reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active **rheumatoid arthritis** (RA) when used in combination with methotrexate
- Inducing and sustaining clinical remission in patients 6 years of age and older with moderately to severely active **ulcerative colitis** (UC) who have had an inadequate response to immunosuppressants such as corticosteroids, azathioprine or 6-mercaptopurine (6-MP).
- **Kevzara**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis** (RA) who have had an inadequate response to one or more disease-modifying anti-rheumatic drugs (DMARDs)
 - Treatment of adult patients with **polymyalgia rheumatica** (PMR) who have had an inadequate response to corticosteroids or who cannot tolerate corticosteroid taper
 - Treatment of patients who weigh 63 kg or greater with active **polyarticular juvenile idiopathic arthritis** (pJIA or JIA)
- **Kineret**
 - Reducing signs and symptoms and slowing the progression of structural damage in moderately to severely active **rheumatoid arthritis** (RA), in patients 18 years of age or older who have failed one or more disease-modifying antirheumatic drugs (DMARDs)
 - Treatment of **neonatal-onset multisystem inflammatory disease** (NOMID)
 - Treatment of **Deficiency of Interleukin-1 Receptor Antagonist** (DIRA)
- **Litfulo**
 - Treatment of severe **alopecia areata** (AA) in adults and adolescents 12 years and older.
- **Olumiant**
 - Treatment of adults with moderate to severely active **rheumatoid arthritis** (RA) who have had an inadequate response to one or more tumor necrosis factor (TNF) blockers
 - Treatment of **coronavirus disease 2019** (COVID-19) in hospitalized adults requiring supplemental oxygen, non-invasive or invasive mechanical ventilation, or extracorporeal membrane oxygenation (ECMO).
 - Treatment of adult patients with severe **alopecia areata** (AA)
- **OmvoH SC, IV**
 - Treatment of moderately to severely active **ulcerative colitis** (UC) in adults
- **Orencia SC**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis** (RA)
 - Treatment of moderately to severely active **polyarticular juvenile idiopathic arthritis** (PJIA or JIA) in pediatric patients 2 years of age and older

	○ Treatment of patients 2 years of age and older with active psoriatic arthritis (PsA) • Orencia IV ○ Treatment of adult patients with moderately to severely active rheumatoid arthritis (RA) ○ Treatment of moderate to severe active juvenile idiopathic arthritis in pediatric patients 6 years of age or older, as monotherapy or concomitantly with methotrexate ○ Treatment of adult patients with active psoriatic arthritis (PsA) ○ Prophylaxis of acute graft versus host disease (aGVHD), in combination with a calcineurin inhibitor and methotrexate, in adults and pediatric patients 2 years of age and older undergoing hematopoietic stem cell transplantation (HSCT) from a matched or 1 allele-mismatched unrelated donor. • Otezla ○ Treatment of adult patients with active psoriatic arthritis (PsA) ○ Treatment of adult patients with plaque psoriasis (PsO) who are candidates for systemic therapy or phototherapy ○ Pediatric patients 6 years of age and older weighing at least 20 kg with moderate to severe plaque psoriasis (PsO) who are candidates for phototherapy or systemic therapy. ○ Treatment of adult patients with oral ulcers associated with Behçet's Disease • Rinvoq ○ Treatment of adult patients with moderately to severely active rheumatoid arthritis (RA) who have had an inadequate response or intolerance to one or more TNF blockers ○ Treatment of adults and pediatric patients 2 years of age and older with active psoriatic arthritis (PsA) who have had an inadequate response or intolerance to one or more TNF blockers. ○ Treatment of adults and pediatric patients 12 years of age and older with refractory, moderate to severe atopic dermatitis (AD) whose disease is not adequately controlled with other systemic drug products, including biologics, or when use of those therapies are inadvisable. ○ Treatment of adults with moderately to severely active ulcerative colitis (UC) who have had an inadequate response or intolerance to one or more TNF blockers. ○ Treatment of adults with moderately to severely active Crohn's disease (CD) who have had an inadequate response or intolerance to one or more TNF blockers. ○ Treatment of adults with active ankylosing spondylitis (AS) who have had an inadequate response or intolerance to one or more TNF blockers. ○ Treatment of adults with active non-radiographic axial spondyloarthritis (nr-axSpA) with objective signs of inflammation who have had an inadequate response or intolerance to TNF blocker therapy ○ Treatment of patients 2 years of age and older with active polyarticular juvenile idiopathic arthritis (PJIA) who have had an inadequate response or intolerance to one or more TNF blockers. • Siliq ○ Treatment of adult patients with moderate to severe plaque psoriasis (PsO) who are candidates for systemic therapy or phototherapy and have failed to respond or have lost response to other systemic therapies • Simponi SC
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- Treatment of adult patients with moderately to severely active **rheumatoid arthritis (RA)** in combination with methotrexate
- Treatment of adult patients with active **ankylosing spondylitis (AS)**
- Treatment of adult patients with active **psoriatic arthritis (PsA)** alone, or in combination with methotrexate
- Treatment of adult patients with moderately to severely active **ulcerative colitis (UC)** who have demonstrated corticosteroid dependence or who have had an inadequate response to or failed to tolerate oral aminosalicylates, oral corticosteroids, azathioprine, or 6-mercaptopurine for inducing and maintaining clinical response, improving endoscopic appearance of the mucosa during induction, inducing clinical remission, and achieving and sustaining clinical remission in induction responders
- **Simponi Aria**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis (RA)** in combination with methotrexate
 - Treatment of active **polyarticular juvenile idiopathic arthritis (pJIA)** in patients 2 years of age and older
 - Treatment of adult patients with active **ankylosing spondylitis (AS)**
 - Treatment of active **psoriatic arthritis (PsA)** in patients 2 years of age or older
- **Skyrizi SC**
 - Treatment of adult patients with moderate-to-severe **plaque psoriasis (PsO)** who are candidates for systemic therapy or phototherapy
 - Treatment of active **psoriatic arthritis (PsA)** in adults
 - Treatment of moderately to severely active **Crohn's disease (CD)** in adults
 - Treatment of moderately to severely active **ulcerative colitis (UC)** in adults
- **Skyrizi IV**
 - Treatment of moderately to severely active **Crohn's disease (CD)** in adults
- **Sotyktu**
 - Treatment of adults with moderate-to-severe **plaque psoriasis (PsO)** who are candidates for systemic therapy or phototherapy.
- **Stelara SC**
 - Treatment of patients 6 years of age or older with active **psoriatic arthritis (PsA)**
 - Treatment of patients 6 years of age or older with moderate to severe **plaque psoriasis (PsO)** who are candidates for systemic therapy or phototherapy
 - Treatment of adult patients with moderately to severely active **Crohn's disease (CD)**
 - Treatment of adult patients with moderately to severely active **ulcerative colitis (UC)**
- **Stelara IV**
 - Treatment of adult patients with moderately to severely active **Crohn's disease (CD)**
 - Treatment of adult patients with moderately to severely active **ulcerative colitis (UC)**
- **Taltz**
 - Treatment of adult patients with active **psoriatic arthritis (PsA)**
 - Treatment of patients 6 years of age or older with moderate to severe chronic **plaque psoriasis (PsO)** who are candidates for systemic therapy or phototherapy
 - Treatment of active **ankylosing spondylitis (AS)**

- Treatment of adult patients with active **non-radiographic axial spondyloarthritis** (nr-axSpA) with objective signs of inflammation
- **Tofidence**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis** (RA) who have had an inadequate response to one or more DMARDs
 - Treatment of adult patients with **giant cell arteritis** (GCA)
 - Treatment of patients 2 years of age and older with active **polyarticular juvenile idiopathic arthritis** (PJIA)
 - Treatment of patients 2 years of age and older with active **systemic juvenile idiopathic arthritis** (SJIA)
 - Treatment of hospitalized adult patients with **coronavirus disease 2019** (COVID-19) who are receiving systemic corticosteroids and require supplemental oxygen, non-invasive or invasive mechanical ventilation, or extracorporeal membrane oxygenation (ECMO)
- **Tremfya SC**
 - Treatment of adult patients with active **psoriatic arthritis** (PsA).
 - Treatment of adult patients with moderate-to-severe **plaque psoriasis** (PsO) who are candidates for systemic therapy or phototherapy
 - Treatment of moderately to severely active **ulcerative colitis** (UC) in adult patients
- **Tremfya IV**
 - Treatment of moderately to severely active **ulcerative colitis** (UC) in adult patients
- **Tyenne**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis** (RA) who have had an inadequate response to one or more DMARDs
 - Treatment of adult patients with **giant cell arteritis** (GCA)
 - Treatment of patients 2 years of age and older with active **polyarticular juvenile idiopathic arthritis** (PJIA)
 - Treatment of patients 2 years of age and older with active **systemic juvenile idiopathic arthritis** (SJIA)
- **Velsipity**
 - Treatment of moderately to severely active **ulcerative colitis** (UC) in adults.
- **Xeljanz**
 - Treatment of adult patients with moderately to severely active **rheumatoid arthritis** (RA) who have had an inadequate response or intolerance to one or more TNF blockers.
 - Xeljanz tablet and Xeljanz oral solution only: Treatment of active **polyarticular course juvenile idiopathic arthritis** (pcJIA) in patients 2 years of age and older who have had an inadequate response or intolerance to one or more TNF blockers.
 - Treatment of adult patients with active **psoriatic arthritis** (PsA) who have had an inadequate response or intolerance to one or more TNF blockers.
 - Treatment of adult patients with moderately to severely active **ulcerative colitis** (UC), who have had an inadequate response or intolerance to one or more TNF blockers.
 - Treatment of adult patients with active **ankylosing spondylitis** (AS) who have had an inadequate response or intolerance to one or more TNF blockers.
- **Zeposia**

	○ Treatment of relapsing forms of multiple sclerosis (MS), to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease, in adults. ○ Treatment of moderately to severely active ulcerative colitis (UC) in adults. • Zymfentra ○ Maintenance treatment of adults with moderately to severely active ulcerative colitis (UC) following treatment with an infliximab product administered intravenously. ○ Maintenance treatment of adults with moderately to severely active Crohn's disease (CD) following treatment with an infliximab product administered intravenously.
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Background:	• Kevzara, Tocilizumab ○ Kevzara and tocilizumab are recombinant humanized interleukin-6 (IL-6) receptor inhibitors that work to inhibit IL-6 mediated actions at soluble and membrane bound IL-6 receptors. Inhibiting the signaling pathway can lead to inhibition of activated T- and B-cells, lymphocytes, monocytes, and fibroblasts. IL-6 is also produced by synovial and endothelial cells which has an effect on the inflammatory process. • Bimzelx, Cosentyx, Siliq, Taltz ○ Bimzelx, Cosentyx, Siliq, and Taltz are human IgG1 monoclonal antibodies. Cosentyx and Taltz selectively bind to the interleukin-17A (IL-17A) cytokine and inhibits its interaction with the IL-17 receptor. Siliq binds with the IL-17 receptor. Bimzelx binds to IL-17A, IL-17F, and IL-17AF. IL-17 is a naturally occurring cytokine that is involved in normal inflammatory and immune responses. • Adalimumab, Enbrel, Cimzia, Infliximab, Simponi, Simponi Aria ○ adalimumab, Cimzia, Enbrel, infliximab, and Simponi are tumor necrosis factor (TNF) inhibitors, which results in an interference in the production of downstream inflammatory mediators, including interleukin-1, prostaglandins, platelet activating factor, and nitric oxide. TNF, a naturally occurring cytokine, mediates inflammation and modulates cellular immune responses. • Otezla ○ Otezla is an oral phosphodiesterase 4 (PDE-4) inhibitor specific for cyclic adenosine monophosphate (cAMP). PDE4 regulates immune and inflammatory processes through control of intracellular cAMP levels and downstream protein kinase A pathways. The production of a number of key inflammatory cytokines is affected by PDE4 including interferon (IFN)γ, tumor necrosis factor (TNF)α, interleukin (IL)-12, and IL-23, thus shaping the immune response. PDE4 inhibition results in increased intracellular cAMP levels and an inhibitory effect on multiple cytokines involved in the inflammatory process. • Stelara ○ Stelara is a human immunoglobulin G (IgG) monoclonal antibody that binds with high affinity and specificity to the p40 subunit, which is part of both interleukin (IL)-12 and IL-23. IL-12 and IL-23 are naturally occurring cytokines that are involved in inflammatory and immune responses, such as natural killer cell activation and CD4+ T-cell differentiation and activation. Stelara binding to the p40 subunit prevents IL-12 and IL-23 from binding to the IL-12 receptor which is the ligand binding subunit of the receptor complexes. Prevention of IL-12 and IL-23 from binding to their respective complexes disrupts IL-12 and IL-23 transduction.
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- **Litfulo, Olumiant, Xeljanz, Rinvoq**
 - Litfulo, Olumiant, Xeljanz, and Rinvoq are an orally bioavailable, small-molecule inhibitors of the Janus kinase (**JAK**) family. These medications modulate the signaling pathway at the point of JAKs, preventing the phosphorylation and activation of Signal Transducers and Activators of Transcription (STATs), which modulate intracellular activity including gene expression.
- **Kineret**
 - Kineret blocks interleukin 1 (**IL-1**) alpha and beta by competitively inhibiting IL-1 binding to the interleukin-1 type I receptor (IL-1RI), which is expressed in a wide variety of tissues and organs. IL-1 production is induced in response to inflammatory stimuli and mediates various physiologic responses including inflammatory and immunological responses.
- **Orencia**
 - Orencia is a fully human recombinant fusion protein categorized as a co-stimulatory or second-signal blocker of **T cell** activation. Orencia disrupts the activation pathway of T cells causing a disturbance in key mechanisms of inflammation and progressive joint destruction.
- **Ilumya, Omvoh, Skyrizi, Tremfya**
 - Ilumya, Omvoh, Skyrizi, and Tremfya are human monoclonal antibodies that selectively antagonize interleukin 23 (**IL-23**) to inhibit the release of pro-inflammatory cytokines and chemokines.
- **Sotyktu**
 - Sotyktu is an inhibitor of tyrosine kinase 2 (**TYK2**), a member of the Janus kinase (JAK) family, and results in inhibition of downstream activation of Signal Transducers and Activators of Transcription (STATs). This results in inhibition of interleukin (IL)-12, IL-23 and type 1 interferon (IFN) pathways which are implicated in multiple immune-mediated diseases.
- **Velsipity, Zeposia**
 - Zeposia is a sphingosine 1-phosphate (**S1P**) receptor modulator that binds with high affinity to S1P receptors 1 and 5, and Velsipity is a S1P receptor that binds with high affinity to S1P receptors 1, 4, and 5. They block the capacity of lymphocytes to egress from lymph nodes, reducing the number of lymphocytes in peripheral blood. The exact mechanism by which S1P receptor modulators exert therapeutic effects in MS (Zeposia only) and UC are unknown, but may involve the reduction of lymphocyte migration into the central nervous system (CNS) and intestine.
- **Entyvio**
 - Entyvio is a humanized monoclonal antibody that specifically binds to the $\alpha 4\beta 7$ **integrin** and blocks the interaction of $\alpha 4\beta 7$ integrin with mucosal addressin cell adhesion molecule-1 (MAdCAM-1) and inhibits the migration of memory T-lymphocytes across the endothelium into inflamed gastrointestinal parenchymal tissue. The interaction of the $\alpha 4\beta 7$ integrin with MAdCAM-1 has been implicated as an important contributor to the chronic inflammation that is a hallmark of inflammatory bowel disease.
- Prescribing Considerations:
 - The member should be under the supervision of a rheumatologist, gastroenterologist, dermatologist, neurologist, or ophthalmologist.

- Litfulo, Xeljanz, Olumiant, and Rinvoq should not be used in combination with other JAK inhibitors, biologic DMARDs, or potent immunosuppressants such as azathioprine or cyclosporine.
- adalimumab, Cimzia, Enbrel, infliximab, Kevzara, Litfulo, Olumiant, Simponi, Simponi Aria, tocilizumab, Xeljanz, and Rinvoq have a black box warning for risk of serious infections leading to hospitalization or death. If a serious infection develops, interrupt the CID product until the infection is controlled. adalimumab, Cimzia, Enbrel, infliximab, Litfulo, Olumiant, Simponi, Simponi Aria, Xeljanz and Rinvoq have a black box warning for lymphoma and other malignancies. Litfulo, Olumiant, Xeljanz and Rinvoq have a black box warning for thrombosis (i.e., deep vein thrombosis, pulmonary embolism, arterial tuberculosis), higher rate of all-cause mortality compared to TNF blockers in RA, including sudden cardiovascular death, and higher rate of major adverse cardiovascular events (MACE) defined as cardiovascular death, myocardial infarction, and stroke) compared to TNF blockers in RA. To note, the black box warnings for Litfulo, Rinvoq, and Olumiant for mortality, malignancy, MACE, and thrombosis were due to the warnings added to Xeljanz and considered to be a JAK inhibitor class warning. Siliq has a black box warning and REMS program for suicidal ideation and behavior. The goal of the Siliq REMS program is to mitigate the observed risk of suicidal ideation and behavior by ensuring that prescribers and patients are educated about the risk of suicidal ideation and behavior. Bimzelx, Cosentyx, Entyvio, Ilumya, Kineret, Omvoh, Orencia, Sotyktu, Stelara, Taltz, and Tremfya do not have any black box warnings.
- While taking a biologic DMARD, the member is currently not using a JAK inhibitor or another biologic DMARD (e.g. Enbrel, Kineret, adalimumab, Cimzia, Orencia, Simponi, Stelara, and tocilizumab).
- Zeposia and Velsipity require assessments prior to initiation including a complete blood count (CBC), electrocardiogram (ECG), liver functions tests, and ophthalmic evaluations (if increased risk of macular edema). Zeposia and Velsipity are contraindicated in patients who have, in the last 6 months, experienced myocardial infarction, unstable angina, stroke, transient ischemic attack, decompensated heart failure requiring hospitalization, or Class III or IV heart failure. Zeposia and Velsipity are also contraindicated in patients with presence of Mobitz type II second-degree or third-degree atrioventricular block, sick sinus syndrome, and sino-atrial block (unless the patient has a functioning pacemaker). Zeposia is contraindicated in severe untreated sleep apnea, or concomitant use of a monoamine oxidase inhibitor.
- When treating Behçet's disease, triamcinolone in Orabase, fluocinonide gel, or clobetasol gel can be utilized as an attempt to control the oral flares as needed without the need for systemic medications that have more side effects and are more expensive. If this is not sufficient, colchicine can be used, recognizing that this once inexpensive medication is now quite costly, and many patients find the associated diarrhea intolerable. Otezla may be a good maintenance therapy option for patients that try and fail either a topical corticosteroid product or colchicine.
- Absolute contraindications to phototherapy (e.g., PUVA, UVB) include dysplastic naevus syndrome, systemic lupus erythematosus, dermatomyositis, genetic skin cancer syndromes, Bloom syndrome, Cockayne syndrome, patients unwilling or unable to comply with safety procedures, and patients who are medically unfit and unable to stand (e.g., severe cardiovascular or respiratory disease).

	○ Absolute contraindications to methotrexate therapy include pregnancy, nursing, alcoholism, alcoholic liver disease or other chronic liver diseases, immunodeficiency syndromes, bone marrow hypoplasia, leukopenia, thrombocytopenia, or significant anemia. History of or current use of greater than moderate alcohol consumption is not a contraindication to methotrexate therapy, but a risk factor for methotrexate-associated hepatotoxicity. ○ Systemic drugs for atopic dermatitis (AD) include monoclonal antibodies administered subcutaneously (SC), such as Dupixent (dupilumab) and Adbry (tralokinumab). Other oral systemic therapy includes off-label use of cyclosporine, methotrexate (MTX), mycophenolate mofetil (MMF), and azathioprine (AZA). Oral immunomodulatory therapy is typically reserved for a subset of patients when topical regimens and/or phototherapy do not adequately control the disease, or when quality of life is substantially impacted. ○ The recommended induction dose of Rinvoq ER tablets for UC is 45 mg once daily for 8 weeks; CD is 45 mg once daily for 12 weeks. The recommended maintenance dosage for UC or CD is 15 mg once daily. A maintenance dosage of 30 mg once daily may be considered for patients with refractory, severe, or extensive UC or CD. ○ Olumiant for treatment of COVID-19 in hospitalized patients should only be administered in an inpatient setting for 14 days or until hospital discharge. Actemra SC administration is not approved for COVID-19. Actemra IV for treatment of COVID-19 should be administered as a single IV infusion, with one additional infusion 8 hours after initial infusion if clinical signs or symptoms worsen or do not improve. Coverage would be under Medicare Part A. Use in the outpatient setting for COVID-19 is not FDA-approved at this time. A drug prescribed to a Part D eligible individual cannot be considered a covered Part D drug if payment for such drug is available under Part A (e.g., hospital inpatient, hospice benefit) or Part B for that individual. https://www.cms.gov/medicare/prescription-drug-coverage/prescriptiondrugcovcontra/downloads/part-d-benefits-manual-chapter-6.pdf ○ A biosimilar is a biological product that is highly similar to and has no clinically meaningful differences in safety, purity and effectiveness from the reference product. A biosimilar may have fewer indications than the innovator due to unexpired patent exclusivity that prevents other manufacturers from obtaining approval for all indications. However, per the FDA clinical activity of a biosimilar is expected to be similar in all clinical settings for which the innovator product has been tested and is approved.
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Approval Criteria

Table 1. Summary of Preferred Products Given by Oral or Subcutaneous (SC) Administration by Indication

	RA	AS	JIA	PsA	PsO	CD	UC	nr-axSpA
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Preferred	• tocilizumab SC² • adalimumab¹ • Enbrel • Rinvoq tablet • Xeljanz tablet/Xeljanz XR tablet	• adalimumab¹ • Cosentyx • Enbrel • Rinvoq tablet • Xeljanz tablet/Xeljanz XR tablet	• tocilizumab SC² • adalimumab¹ • Enbrel • Rinvoq tablet/Rinvoq LQ • Xeljanz tablet/oral solution	• adalimumab¹ • Cosentyx • Enbrel • Otezla • Rinvoq tablet/Rinvoq LQ • Skyrizi SC • Stelara SC • Xeljanz tablet/Xeljanz XR tablet	• adalimumab¹ • Cosentyx • Enbrel • Otezla • Skyrizi SC • Stelara SC	• adalimumab¹ • Rinvoq tablet • Skyrizi SC • Stelara SC	• adalimumab¹ • Rinvoq tablet • Skyrizi SC • Stelara SC • Xeljanz tablet/Xeljanz XR tablet	• Cimzia • Cosentyx • Rinvoq tablet
Non-preferred (must try TWO preferred agents)	• Cimzia • Kevzara • Kineret • Olumiant • Orencia SC • Simponi SC (50 mg)	• Cimzia • Simponi SC (50 mg) • Taltz	• Kevzara • Orencia SC	• Cimzia • Simponi SC (50 mg) • Orencia SC • Taltz • Tremfya	• Bimzelx • Cimzia • Ilumya • Siliq • Taltz • Tremfya • Sotyktu	• Cimzia • Entyvio SC • Zymfentra	• Entyvio SC • Omvoh • Simponi SC (100 mg) • Tremfya • Velsipity • Zeposia • Zymfentra	• Taltz

¹ A trial of any preferred adalimumab product counts as one (1) adalimumab preferred product. See Table 2 for preferred adalimumab biosimilars.

² A trial of any preferred tocilizumab product counts as one (1) tocilizumab preferred product. See Table 2 for preferred tocilizumab biosimilars.

Table 2. Summary of Preferred Biosimilars

Drug Name	Preferred	Non-Preferred
adalimumab	• Humira [00074] labeler • Cyltezo • Yuflyma	• Abrilada • adalimumab-aacf • adalimumab-aaty • adalimumab-adaz • adalimumab-adbm • adalimumab-fkjp • adalimumab-ryvk • Amjevita • Idacio • Hadlima • Hulio • Humira [83457] labeler • Hyrimoz • Simlandi • Yusimry
infliximab IV	• Avsola • Inflectra	• Infliximab • Remicade • Renflexis
tocilizumab SC	• Actemra	• Tyenne
tocilizumab IV	• Actemra	• Tofidence • Tyenne

- A request for a non-preferred adalimumab product should be directed to two (2) preferred adalimumab products.
- A request for a non-preferred infliximab IV product should be directed to two (2) preferred infliximab IV products.
- A request for a non-preferred tocilizumab SC or IV product should be directed to Actemra.
- Any biosimilars not listed are non-preferred until indicated otherwise.

- Humira prior authorization should be entered at the NDC level. Multiple NDCs for a preferred labeler can be entered upon authorization.

I. adalimumab (See Table 2 for Preferred Products)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- d. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

2. Ankylosing Spondylitis (AS)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- d. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

3. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of moderate to severe JIA.
- c. The member meets one (1) of the following criteria **(i. or ii.)**:
 - i. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
 - ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- d. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

4. Psoriatic Arthritis (PsA)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.
- c. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

5. Plaque Psoriasis (PsO)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g. methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

6. Crohn's Disease (CD)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 6 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.
- c. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

7. Ulcerative Colitis (UC)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a., b. and c.)**:

- a. The member is 5 years of age or older.
- b. The member has a diagnosis of moderate or severe UC.
- c. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

8. Hidradenitis Suppurativa

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 12 years of age or older.
- b. The member has a diagnosis of moderate to severe HS.
- c. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

9. Uveitis (UV)

When a benefit, coverage of an adalimumab product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 2 years of age or older
- b. The member has a diagnosis of non-infectious intermediate, posterior or panuveitis.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) immunosuppressant (e.g., azathioprine, 6-mercaptopurine) or corticosteroid, or all are contraindicated.
- d. If the request is for a non-preferred adalimumab product, the member has experienced therapeutic failure or intolerance to two (2) preferred adalimumab products.

B. Quantity Limitations

When prior authorization is approved, adalimumab prefilled syringes, pens, or auto-injectors may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
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RA, AS, PsA	N/A	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks
RA*	N/A	Four (4) prefilled pen/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks (Without concurrent methotrexate therapy) -OR- Two (2) prefilled pens/syringes (80 mg/0.8 mL) every four (4) weeks (Without concurrent methotrexate therapy)
JIA, pediatric UV	N/A	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks
chronic PsO	Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks [†]
adult CD	Six (6) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- Three (3) prefilled pens/syringes (80 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks [†]
pediatric CD (37 lbs - 88 lbs)	Three (3) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (20 mg/0.2 mL or 20 mg/0.4 mL) every four (4) weeks -OR- Four (4) prefilled pens/syringes (20 mg/0.2 mL or 20 mg/0.4 mL) every four (4) weeks [†]
pediatric CD (≥ 88 lbs)	Six (6) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks

	-OR- Three (3) prefilled pens/syringes (80 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	-OR- Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks[†]
adult UC	Six (6) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- Three (3) prefilled pens/syringes (80 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks[†]
pediatric UC (44 lbs - 88 lbs)	Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Four (4) prefilled syringes (20 mg/0.4 mL or 20 mg/0.2 mL) every four (4) weeks^{††}
pediatric UC (≥ 88 lbs)**	Eight (8) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- Four (4) prefilled pens/syringes (80 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Two (2) prefilled pens/syringes (80 mg/0.8 mL) every four (4) weeks
HS in adults and adolescents (12 years and older) ≥ 60 kg*	Seven (7) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- Four (4) prefilled pens/syringes	Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks -OR- Two (2) prefilled pens/syringes (80 mg/0.8 mL) every four (4) weeks

	(80 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	
HS in adolescents (12 years and older) 30 kg to < 60 kg*	Five (5) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks
adult UV	Four (4) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) within the first four (4) weeks of therapy -OR- One (1) starter package kit	Two (2) prefilled pens/syringes (40 mg/0.4 mL or 40 mg/0.8 mL) every four (4) weeks

N/A=not applicable

- *Patients diagnosed with rheumatoid arthritis (without concurrent methotrexate therapy), hidradenitis suppurativa, pediatric UC (≥ 88 lbs) may receive weekly dosing of adalimumab prefilled syringes. Patient Level Authorization (PLA) input - Retail: 4 pens/syringes; Mail: 12 pens/syringes
- ** Continuation of pediatric UC dosing in patients who turn 18 years of age may be approved when documentation of stability or beneficial response to therapy.
- Coding of quantity level limitations is at the maintenance therapy threshold except for rheumatoid arthritis and hidradenitis suppurativa weekly dosing administration.
- †Four (4) prefilled syringes every four (4) weeks may be approved when there is clinical documentation that treatment with two (2) prefilled syringes every four (4) weeks was ineffective.
- †† Only adalimumab 20 mg prefilled syringe may be approved for every week dosing.
- Adalimumab starter packs have a quantity limit of 1 kit per 274 days but can be billed as a quantity/dispensing size of 2, 3, 4, or 6 pens or syringes depending on indication.

II. **Bimzelx (bimekizumab-bkzx)**

A. **Approval Criteria**

1. **Plaque Psoriasis (PsO)**

When a benefit, coverage of Bimzelx may be approved when all of the following criteria are met (**a. through d.**):

- The member is 18 years of age or older.
- The member has a diagnosis of moderate to severe PsO.
- The member meets one (1) of the following criteria (**i., ii., or iii.**):
 - The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - The member is contraindicated to both phototherapy and systemic therapy.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**).

Dosing

In addition to the approval criteria outlined above, documentation that member weight and prescribed Bimzelx dose is consistent with dosing below:

- **PsO**
 - a. < 120 kg (264 lbs): 320 mg initially and every 4 weeks for the first 16 weeks, followed by 320 mg every 8 weeks.
 - b. ≥ 120 kg (264 lbs) (**a. or b.**):
 - a. 320 mg initially and every 4 weeks for the first 16 weeks, followed by 320 mg every 8 weeks.
 - b. 320 mg initially and every 4 weeks for the first 16 weeks, followed by 320 mg every 4 weeks.

B. Quantity Limitations

When prior authorization is approved, Bimzelx may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
PsO (< 120 kg) ¹	Ten (10) autoinjectors/prefilled syringes within the first sixteen (16) weeks of therapy	Two (2) autoinjectors/prefilled syringes every eight (8) weeks
PsO (≥ 120 kg) ¹	Ten (10) autoinjectors/prefilled syringes within the first sixteen (16) weeks of therapy	Two (2) autoinjectors/prefilled syringes every four (4) weeks -OR- Two (2) autoinjectors/prefilled syringes every eight (8) weeks

¹ See dosing criteria

III. Cimzia (certolizumab)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a. through d.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of RA (see **Table 1**).

2. Ankylosing Spondylitis (AS)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a. through d.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of AS (see **Table 1**).

3. Psoriatic Arthritis (PsA)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.

- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsA (see **Table 1**).

4. Crohn's Disease (CD)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of CD (see **Table 1**).

5. Plaque Psoriasis (PsO)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a. through d.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria (**i., ii., or iii.**):
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**).

6. Non-radiographic Axial Spondyloarthritis (nr-axSpA)

When a benefit, coverage of Cimzia may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of nr-axSpA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) nonsteroidal anti-inflammatory drugs (NSAIDs), or all NSAIDs are contraindicated.

B. Quantity Limitations

When prior authorization is approved, Cimzia may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA, AS, PsA, CD, nr-axSpA	Ten (10) syringes in first twelve (12) weeks of therapy -OR- One (1) starter package kit	Two (2) syringes every four (4) weeks
PsO	Six (6) syringes in first four (4) weeks of therapy	One (1) syringe or two (2) syringes every two (2) weeks

- To note, starter package kits are coded for quantity level limitations of one (1) kit per 365 days.
- Coding of quantity level limitations is at the maintenance therapy threshold except for PsO

IV. Cosentyx (secukinumab)

A. Approval Criteria for Cosentyx SC

1. Ankylosing Spondylitis (AS)

When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.

- b. The member has a diagnosis of AS.
 - c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- 2. Psoriatic Arthritis (PsA)**
When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met **(a. and b.)**:
- a. The member is 2 years of age or older.
 - b. The member has a diagnosis of PsA.
- 3. Plaque Psoriasis (PsO), including Scalp Psoriasis**
When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met **(a., b., and c.)**:
- a. The member is 6 years of age or older.
 - b. The member has a diagnosis of moderate to severe PsO.
 - c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- 4. Non-radiographic Axial Spondyloarthritis (nr-axSpA)**
When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met **(a., b., and c.)**:
- a. The member is 18 years of age or older.
 - b. The member has a diagnosis of nr-axSpA.
 - c. The member has experienced therapeutic failure or intolerance to at least two (2) nonsteroidal anti-inflammatory drugs (NSAIDs), or all NSAIDs are contraindicated.
- 5. Entesitis-Related Arthritis (ERA)**
When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met **(a. and b.)**:
- a. The member is 4 years of age or older.
 - b. The member has a diagnosis of active ERA.
- 6. Hidradenitis Suppurativa (HS)**
When a benefit, coverage of Cosentyx SC may be approved when all of the following criteria are met **(a. and b.)**:
- a. The member is 18 years of age or older.
 - b. The member has a diagnosis of moderate to severe HS.

B. Quantity Limitations for Cosentyx SC

When prior authorization is approved, Cosentyx SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
AS	One (1) or Five (5) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL or 300 mg/2mL) or Two (2) (150 mg/mL) pens/prefilled syringes every four (4) weeks
Adult PsA⁴	One (1) or Five (5) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL or 300 mg/2mL) or Two (2) (150 mg/mL) pens/prefilled syringes every four (4) weeks

ERA and Pediatric PsA (≥ 50 kg)^{1,2}	Five (5) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL) pen/prefilled syringe every four (4) weeks
ERA and Pediatric PsA (≥ 15 kg to < 50 kg)¹	Five (5) (75 mg/mL) prefilled syringes within the first four (4) weeks of therapy	One (1) (75mg/mL) prefilled syringe every four (4) weeks
Adult PsO with or without PsA, scalp PsO^{1,3}	Five (5) (300 mg/2mL) or Ten (10) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL or 300 mg/2mL) or Two (2) (150 mg/mL) pens/prefilled syringes every four (4) weeks
Pediatric PsO (≥ 50 kg)^{1,2}	Five (5) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL) pen/prefilled syringe every four (4) weeks
Pediatric PsO (< 50 kg)¹	Five (5) (75 mg/mL) prefilled syringes within the first four (4) weeks of therapy	One (1) (75mg/mL) prefilled syringe every four (4) weeks
nr-axSpA	One (1) or Five (5) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (150 mg/mL) pen/prefilled syringe every four (4) weeks
HS⁵	Five (5) (300 mg/2mL) or Ten (10) (150 mg/mL) pens/prefilled syringes within the first four (4) weeks of therapy	One (1) (300 mg/mL) pen/prefilled syringe or Two (150 mg/mL) pens/prefilled syringes every four (4) weeks

¹ Pediatric patients with PsO or PsA may require 3 pens/prefilled syringes within a four (4) week time frame if switching from pediatric dosing (75mg/mL) to adult dosing (300mg/2mL).

² Pediatric patients with PsO, PsA, and ERA (≥ 50 kg) are subject to dose optimization of one (1) pen/prefilled syringe (150 mg/mL) every four (4) weeks.

³ Adult patients < 50 kg with PsO are subject to adult dosing recommendations and quantity limits.

⁴ For adult PsA patients with coexistent moderate to severe PsO, use the dosing for adult PsO.

⁵ Two (2) (300 mg/mL) pens/prefilled syringes or Four (150 mg/mL) pens/prefilled syringes every four (4) weeks may be approved when there is clinical documentation that treatment with 300 mg every 4 weeks was ineffective.

C. Approval Criteria for Cosentyx IV

1. Ankylosing Spondylitis (AS)

When a benefit, coverage of Cosentyx IV may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of AS.
- c.** The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.

2. Psoriatic Arthritis (PsA)

When a benefit, coverage of Cosentyx IV may be approved when all of the following criteria are met **(a. and b.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of PsA.

3. Non-radiographic Axial Spondyloarthritis (nr-axSpA)

When a benefit, coverage of Cosentyx IV may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of nr-axSpA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) nonsteroidal anti-inflammatory drugs (NSAIDs), or all NSAIDs are contraindicated.

D. Quantity Limitations for Cosentyx IV

When prior authorization is approved, Cosentyx IV may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
AS, PsA, nr-axSpA	One (1) induction course override allowed within the first week of therapy if requested	3 vials (125 mg/5 mL) every four (4) weeks

V. Enbrel (etanercept)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.

2. Ankylosing Spondylitis (AS)

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.

3. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of JIA.
- c. The member meets one (1) of the following criteria **(i. or ii.)**:
 - i. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
 - ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.

4. Psoriatic Arthritis (PsA)

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a. and b.)**:

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of PsA.

5. Plaque Psoriasis (PsO), adults

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.

6. Plaque Psoriasis (PsO), pediatrics

When a benefit, coverage of Enbrel may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is ≥ 4 but < 18 years of age.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.

B. Quantity Limitations

When prior authorization is approved, Enbrel may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
AS, JIA, pediatric PsO, PsA, RA	N/A	Four (4) 50 mg prefilled syringes/autoinjectors every four (4) weeks -OR- Eight (8) 25 mg prefilled syringes/ autoinjectors every four (4) weeks
PsO, adults	Twenty-four (24) 50 mg syringes/ autoinjectors within the first twelve (12) weeks of therapy -OR- Forty-eight (48) 25 mg syringes/ autoinjectors within the first twelve (12) weeks of therapy	Four (4) 50 mg prefilled syringes/ autoinjectors every four (4) weeks -OR- Eight (8) 25 mg prefilled syringes/ autoinjectors every four (4) weeks

N/A=not applicable

VI. Entyvio (vedolizumab) SC

A. Approval Criteria

1. Ulcerative Colitis (UC)

When a benefit, coverage of Entyvio SC may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).

- d. The prescriber attests the member has already received at least two doses of Entyvio IV at least 6 weeks before initiating therapy with Entyvio SC, or is currently undergoing induction therapy with Entyvio IV.

2. Crohn's Disease (CD)

When a benefit, coverage of Entyvio SC may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of CD (see **Table 1**).
- d. The prescriber attests the member has already received at least two doses of Entyvio IV at least 6 weeks before initiating therapy with Entyvio SC, or is currently undergoing induction therapy with Entyvio IV.

B. Quantity Limitations

When prior authorization is approved, Entyvio SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
UC, CD	N/A	Two (2) prefilled syringes/pens every four (4) weeks

VII. Entyvio (vedolizumab) IV

A. Approval Criteria

1. Ulcerative Colitis (UC)

When a benefit, coverage of Entyvio IV may be approved when all of the following criteria are met **(a. and b.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.

2. Crohn's Disease (CD)

When a benefit, coverage of Entyvio IV may be approved when all of the following criteria are met **(a. and b.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.

VIII. Ilumya (tildrakizumab-asmn)

A. Approval Criteria

1. Plaque Psoriasis (PsO)

When a benefit, coverage of Ilumya may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).

- iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**)

B. Quantity Limitations

When prior authorization is approved, Ilumya may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
PsO	Two (2) syringes (100 mg/mL) within the first four (4) weeks of therapy	One (1) prefilled syringe per twelve (12) weeks

IX. Infliximab IV or Infliximab IV Biosimilars (See Table 2 for Preferred Products)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe active RA.
- c. The member will be using infliximab in combination with methotrexate.
- d. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.)**:
 - i. Avsola
 - ii. Inflectra

2. Psoriatic Arthritis (PsA)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.
- c. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.)**:
 - i. Avsola
 - ii. Inflectra

3. Ankylosing Spondylitis (AS)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- d. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.)**:
 - i. Avsola
 - ii. Inflectra

4. Crohn's Disease (CD)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member is 6 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.
- c. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.):**
 - i. Avsola
 - ii. Inflectra

5. Ulcerative Colitis (UC)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member is 6 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.):**
 - i. Avsola
 - ii. Inflectra

6. Plaque Psoriasis (PsO)

When a benefit, coverage of an infliximab IV product may be approved when all of the following criteria are met **(a. through d.):**

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.):**
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g. methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. If the request is for Infliximab, Remicade, or Renflexis, the member has experienced therapeutic failure or intolerance to all of the following agents **(i. and ii.):**
 - i. Avsola
 - ii. Inflectra

X. Kevzara (sarilumab)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Kevzara may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of RA (see **Table 1**).

2. Polymyalgia Rheumatica (PMR)

When a benefit, coverage of Kevzara may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PMR.
- c. The member meets one (1) of the following **(i., ii., or iii.):**

- i. The member will be using Kevzara in combination with a tapering course of corticosteroids.
- ii. The member has experienced an inadequate response to corticosteroids.
- iii. The member has experienced an intolerance to corticosteroid taper.

3. Juvenile Idiopathic Arthritis (JIA)

When a benefit coverage of Kevzara may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member weighs greater than or equal to 63 kg.
- b. The member has a diagnosis of JIA.
- c. The member meets one (1) of the following criteria **(i. or ii.)**:
 - i. The member has experienced therapeutic failure or intolerance to at least one (1) nonbiologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all nonbiologic DMARDs are contraindicated.
 - ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of JIA (see **Table 1**).

B. Quantity Limitations

When prior authorization is approved, Kevzara may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA	N/A	Two (2) prefilled pens/syringes (150 mg/1.14 mL or 200 mg/1.14 mL) every four (4) weeks
PMR	N/A	Two (2) prefilled pens/syringes (200 mg/1.14 mL) every four (4) weeks
JIA	N/A	Two (2) prefilled syringes (200 mg/1.14 mL) every four (4) weeks

N/A=not applicable

XI. Kineret (anakinra)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Kineret may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of RA (see **Table 1**).

2. Neonatal-Onset Multisystem Inflammatory Disease (NOMID)

When a benefit, coverage of Kineret may be approved when the following criterion is met **(a.)**:

- a.** The member has a diagnosis of NOMID.

3. Deficiency of Interleukin-1 Receptor Antagonist (DIRA)

When a benefit, coverage of Kineret may be approved when all of the following criteria are met **(a. and b.)**:

- a.** The member has a diagnosis of DIRA.
- b.** The member has experienced therapeutic failure or intolerance to at least one (1) corticosteroid, or all corticosteroids are contraindicated.

Dosing

In addition to the approval criteria outlined above, documentation that member weight and prescribed Kineret dose is consistent with dosing below:

***NOMID/DIRA:** The recommended starting dose is 1-2 mg/kg daily. The dose can be individually adjusted to a maximum of 8 mg/kg daily. Kineret may be divided into twice daily dosing. A new syringe must be used for each dose and any unused portion after each dose should be discarded.

B. Quantity Limitations

When prior authorization is approved, Kineret may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA, NOMID*, DIRA*	One (1) prefilled syringe once daily	One (1) prefilled syringe once daily

XII. Litfulo (ritlecitinib)

A. Approval Criteria

1. Alopecia Areata (AA)

When a benefit, coverage of Litfulo may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 12 years of age or older.
- b.** The member has a diagnosis of severe AA.
- c.** The member has experienced therapeutic failure or intolerance to one (1) of the following, or contraindication to all **(i. or ii.)**:
 - i.** Intralesional corticosteroid (e.g., methylprednisolone, triamcinolone)
 - ii.** High potency topical corticosteroid (e.g., betamethasone dipropionate)

XIII. Olumiant (baricitinib)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Olumiant may be approved when all of the following criteria are met **(a. through d.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severely active RA.
- c.** The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- d.** The member has experienced therapeutic failure or intolerance to least two (2) preferred biologic products for the treatment of RA (see **Table 1**), with at least one (1) being a

tumor necrosis factor (TNF) blocker.

2. Alopecia Areata (AA)

When a benefit, coverage of Olumiant may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of severe AA.
- c. The member has experienced therapeutic failure or intolerance to one (1) of the following, or contraindication to all **(i. or ii.)**:
 - i. Intralesional corticosteroid (e.g., methylprednisolone, triamcinolone)
 - ii. High potency topical corticosteroid (e.g., betamethasone dipropionate)

XIV. OmvoH (mirikizumab-mrkz)

A. Approval Criteria

1. Ulcerative Colitis (UC)

When a benefit, coverage of OmvoH may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).
- d. If the request is for OmvoH SC, the prescriber attests the member has already received three (3) induction doses of OmvoH IV within 3 months of initiating therapy with OmvoH SC or is currently undergoing induction therapy with OmvoH IV.

B. Quantity Limitations

When prior authorization is approved, OmvoH IV and SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
UC	Three (3) single-dose vials within the first twelve (12) weeks of therapy	Two (2) prefilled pens every four (4) weeks

XV. Orencia (abatacept)

A. Approval Criteria for Orencia SC

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Orencia SC may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of RA (see **Table 1**).

2. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Orencia SC may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of JIA.

- c. The member meets one (1) of the following criteria (**i. or ii.**):
 - i. The member has experienced therapeutic failure or intolerance to at least one (1) nonbiologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all nonbiologic DMARDs are contraindicated.
 - ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of JIA (see **Table 1**).

3. Psoriatic Arthritis (PsA)

When a benefit, coverage of Orencia SC may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of PsA.
- c. If the member is 18 years of age or older, the member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsA (see **Table 1**).

B. Quantity Limitations

When prior authorization is approved, Orencia SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA, JIA, PsA	N/A	Four (4) prefilled syringes every four (4) weeks

C. Approval Criteria for Orencia IV

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Orencia IV may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to one (1) preferred intravenous biologic product (i.e., infliximab, Simponi Aria).

2. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Orencia IV may be approved when all of the following criteria are met (**a. and b.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of polyarticular JIA.

3. Psoriatic Arthritis (PsA)

When a benefit, coverage of Orencia IV may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has diagnosis of PsA.
- c. The member has experienced therapeutic failure or intolerance to the preferred intravenous biologic product (i.e., infliximab).

4. Acute Graft Versus Host Disease (aGVHD)

When a benefit, coverage of Orencia IV may be approved when all of the following criteria are met (**a. through d.**):

- a. The member is 2 years of age or older.

- b. The member is using Orencia IV for prophylaxis of aGVHD.
- c. The member is undergoing hematopoietic stem cell transplantation (HSCT) from a matched or 1 allele-mismatched unrelated donor.
- d. The member will be using Orencia IV in combination with all of the following **(i. and ii.)**:
 - i. calcineurin inhibitor (e.g., tacrolimus, cyclosporine)
 - ii. methotrexate

XVI. Otezla (apremilast)

A. Approval Criteria

1. Psoriatic Arthritis (PsA)

When a benefit, coverage of Otezla may be approved when all of the following criteria are met **(a. and b.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.

2. Adult Plaque Psoriasis (PsO)

When a benefit, coverage of Otezla may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g. methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.

3. Pediatric Plaque Psoriasis (PsO)

When a benefit, coverage of Otezla may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is ≥ 6 years and < 18 of age or older.
- b. The member weighs ≥ 20 kg.
- c. The member has a diagnosis of moderate to severe PsO.
- d. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g. methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.

4. Oral Ulcers associated with Behçet's Disease

When a benefit, coverage of Otezla may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of oral ulcers associated with Behçet's Disease.
- c. The member has experienced therapeutic failure or intolerance to one (1) systemic therapy (e.g., colchicine, systemic corticosteroid, azathioprine) for prevention of recurrent oral ulcers.

XVII. Rinvoq (upadacitinib)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Rinvoq ER tablet may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member has a diagnosis of moderate to severe RA.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

2. Psoriatic Arthritis (PsA)

When a benefit, coverage of Rinvoq ER tablet or Rinvoq LQ may be approved when all of the following criteria are met **(a., b., c.):**

- a. The member has a diagnosis of PsA.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.
- c. If the member is 18 years of age or older, the member is requesting Rinvoq ER tablets.

3. Atopic Dermatitis (AD)

When a benefit, coverage of Rinvoq ER tablet may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member has a diagnosis of atopic dermatitis, classified as all of the following **(i. and ii.):**
 - i. Moderate-to-severe
 - ii. Refractory
- b. The member meets one (1) of the following criteria **(i. or ii.):**
 - i. The member has experienced therapeutic failure or intolerance to one (1) of the following **(A) or B))**:
 - A)** One (1) generic topical corticosteroid.
 - B)** One (1) generic topical calcineurin inhibitor (i.e., tacrolimus, pimecrolimus)
 - ii. The prescriber attests the member has severe atopic dermatitis and topical therapy would not be advisable for maintenance therapy as evidenced by one (1) of the following **(A) or B))**:
 - A)** The member is incapable of applying topical therapies due to the extent of body surface area (BSA) involvement.
 - B)** Topical therapies are contraindicated due to severely damaged skin.
- c. The member has experienced therapeutic failure or intolerance to one (1) systemic therapy for atopic dermatitis, or all systemic therapies are contraindicated.

4. Ulcerative Colitis (UC)

When a benefit, coverage of Rinvoq ER tablet may be approved when all of the following criteria are met **(a. and b.):**

- a. The member has a diagnosis of moderate to severe UC.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

5. Ankylosing Spondylitis (AS)

When a benefit, coverage of Rinvoq ER tablet be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member has a diagnosis of AS.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

6. Non-radiographic Axial Spondyloarthritis (nr-axSpA)

When a benefit, coverage of Rinvoq ER tablet may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member has a diagnosis of nr-axSpA.
- b. The member has experienced therapeutic failure or intolerance to at least two (2) nonsteroidal anti-inflammatory drugs (NSAIDs), or all NSAIDs are contraindicated.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

7. Crohn's Disease (CD)

When a benefit, coverage of Rinvoq ER tablet may be approved when all of the following criteria are met **(a. and b.):**

- a. The member has a diagnosis of moderate to severe CD.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

8. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Rinvoq ER tablet or Rinvoq LQ may be approved when all of the following criteria are met **(a., b., and c.):**

- a. The member has a diagnosis of JIA.
- b. The member meets one (1) of the following criteria **(i. or ii.):**
 - i. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
 - ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

XVIII. Siliq (brodalumab)

A. Approval Criteria

1. Plaque Psoriasis (PsO)

When a benefit, initiation of Siliq may be approved when all of the following criteria are met **(a. through d.):**

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.):**
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g., PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**).

B. Quantity Limitations

When prior authorization is approved, Siliq may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
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PsO	Three (3) prefilled syringes within the first two (2) weeks of therapy	Two (2) prefilled syringes every four (4) weeks
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XIX. Simponi (golimumab)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Simponi SC may be approved when all of the following criteria are met **(a. through e.)**:

- The member is 18 years of age or older.
- The member has a diagnosis of moderate to severe RA.
- The member will use Simponi 50 mg in combination with methotrexate.
- The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all nonbiologic DMARDs are contraindicated.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of RA (see **Table 1**).

2. Ankylosing Spondylitis (AS)

When a benefit, coverage of Simponi SC may be approved when all of the following criteria are met **(a. through d.)**:

- The member is 18 years of age or older.
- The member has a diagnosis of AS.
- The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of AS (see **Table 1**).

3. Psoriatic Arthritis (PsA)

When a benefit, coverage of Simponi SC 50 mg may be approved when all of the following criteria are met **(a., b., and c.)**:

- The member is 18 years of age or older.
- The member has a diagnosis of PsA.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsA (see **Table 1**).

4. Ulcerative Colitis (UC)

When a benefit, coverage of Simponi SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- The member is 18 years of age or older.
- The member has a diagnosis of moderate to severe UC.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).

B. Quantity Limitations

When prior authorization is approved, Simponi SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA, AS, PsA	One (1) 50 mg syringe/autoinjector within the first four (4) weeks of therapy	One (1) 50 mg syringe/autoinjector every four (4) weeks

UC	Three (3) 100 mg syringes/ autoinjector within the first four (4) weeks of therapy	One (1) 100 mg syringe/ autoinjector every four (4) weeks
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XX. Simponi Aria (golimumab)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Simponi Aria may be approved when all of the following criterion is met **(a. and b.)**:

- a.** The member has a diagnosis of moderate to severe RA.
- b.** The member will use Simponi Aria in combination with methotrexate.

2. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Simponi Aria may be approved when all of the following criteria are met **(a. and b.)**:

- a.** The member is 2 years of age or older.
- b.** The member has a diagnosis of JIA.

3. Ankylosing Spondylitis (AS)

When a benefit, coverage of Simponi Aria may be approved when the following criterion is met **(a. and b.)**:

- a.** The member has a diagnosis of AS.
- b.** The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.

4. Psoriatic Arthritis (PsA)

When a benefit, coverage of Simponi Aria may be approved when the following criterion is met **(a.)**:

- a.** The member has a diagnosis of PsA.

XXI. Skyrizi (risankizumab)

A. Approval Criteria

1. Plaque Psoriasis (PsO)

When a benefit, coverage of Skyrizi SC pen/syringe may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe PsO.
- c.** The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i.** The member has experienced therapeutic failure or intolerance to phototherapy (e.g., PUVA, UVB).
 - ii.** The member has experienced therapeutic failure or intolerance to systemic therapy (e.g., methotrexate).
 - iii.** The member is contraindicated to both phototherapy and systemic therapy.

2. Psoriatic Arthritis (PsA)

When a benefit, coverage of Skyrizi SC pen/syringe may be approved when all of the following criteria are met **(a. and b.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of PsA.

3. Crohn's Disease (CD)

When a benefit, coverage of Skyrizi IV or SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe CD.
- c. If the request is for Skyrizi SC cartridge with on-body injector, the prescriber attests the member has already received three (3) induction doses of Skyrizi IV within 3 months of initiating therapy with Skyrizi SC cartridge with on-body injector or is currently undergoing induction therapy with Skyrizi IV.

4. Ulcerative Colitis (UC)

When a benefit, coverage of Skyrizi IV or SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. If the request is for Skyrizi SC cartridge with on-body injector, the prescriber attests the member has already received three (3) induction doses of Skyrizi IV within 3 months of initiating therapy with Skyrizi SC cartridge with on-body injector or is currently undergoing induction therapy with Skyrizi IV

B. Quantity Limitations

When prior authorization is approved, Skyrizi IV or SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
PsO, PsA	Two (2) prefilled syringes/pens (150 mg/mL) within the first four (4) weeks of therapy	One (1) prefilled syringe/pen (150 mg/mL) every twelve (12) weeks
CD	Three (3) vials (600 mg/10 mL) within the first twelve (12) weeks of therapy	One (1) prefilled cartridge with on-body injector every eight (8) weeks
UC	Six (6) vials (600 mg/10mL) within the first twelve (12) weeks of therapy	One (1) prefilled cartridge with on-body injector every eight (8) weeks

XXII. Sotyktu (deucravacitinib)

A. Approval Criteria

1. Plaque Psoriasis (PsO)

When a benefit, coverage of Sotyktu may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g. PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**).

XXIII. Stelara (ustekinumab)

A. Approval Criteria

1. Psoriatic Arthritis (PsA)

When a benefit, coverage of Stelara SC may be approved when the following criteria are met **(a. and b.)**:

- a.** The member is 6 years of age or older.
- b.** The member has a diagnosis of PsA.

2. Plaque Psoriasis (PsO)

When a benefit, coverage of Stelara SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 6 years of age or older.
- b.** The member has a diagnosis of moderate to severe PsO.
- c.** The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i.** The member has experienced therapeutic failure or intolerance to phototherapy (e.g., PUVA, UVB).
 - ii.** The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii.** The member is contraindicated to both phototherapy and systemic therapy.

3. Crohn's Disease (CD)

When a benefit, coverage of Stelara may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe CD.
- c.** If the request is for Stelara SC, the prescriber attests the member has already received a single induction dose of Stelara IV within 2 months of initiating therapy with Stelara SC or is currently undergoing induction therapy with Stelara IV.

4. Ulcerative Colitis (UC)

When a benefit, coverage of Stelara may be approved for UC when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe UC.
- c.** If the request is for Stelara SC, the prescriber attests the member has already received a single induction dose of Stelara IV within 2 months of initiating therapy with Stelara SC or is currently undergoing induction therapy with Stelara IV.

Dosing

In addition to the approval criteria outlined above, documentation that member weight and prescribed Stelara dose is consistent with dosing below:

- **PsO (18 years of age or older)**
 - a.** ≤ 100 kg (220 lbs): 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
 - b.** > 100 kg (220 lbs): 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- **PsO (6 to 17 years of age)**
 - a.** < 60 kg (132 lbs): 0.75 mg/kg initially and 4 weeks later, followed by 0.75 mg/kg every 12 weeks.
 - b.** 60 to 100 kg (220 lbs): 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
 - c.** > 100 kg (220 lbs): 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- **PsA (18 years of age or older)**
 - a.** 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
 - b.** For patients with co-existent moderate-to-severe plaque psoriasis weighing > 100 kg (220 lbs): 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- **PsA (6 to 17 years of age)**

- a. < 60 kg (132 lbs): 0.75 mg/kg initially and 4 weeks later, followed by 0.75 mg/kg every 12 weeks.
- b. 60 to 100 kg (220 lbs): 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
- c. For patients with co-existent moderate-to-severe plaque psoriasis weighing > 100 kg (220 lbs): 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- **CD**
 - a. Following a weight-based intravenous dose, the recommended dose is 90 mg 8 weeks following the intravenous dose, then every 8 weeks thereafter.
- **UC**
 - a. Following a weight-based intravenous dose, the recommended dose is 90 mg 8 weeks following the intravenous dose, then every 8 weeks thereafter.

B. Quantity Limitations

When prior authorization is approved, Stelara IV and SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
PsA, PsO	Two (2) prefilled syringes within the first four (4) weeks of therapy	One (1) syringe every twelve (12) weeks
CD, UC (≤ 55 kg)	Two (2) vials (130 mg/26 mL) within the first eight (8) weeks of therapy	One (1) syringe (90 mg/mL) every eight (8) weeks
CD, UC (> 55 kg to 85 kg)	Three (3) vials (130 mg/26 mL) within the first eight (8) weeks of therapy	One (1) syringe (90 mg/mL) every eight (8) weeks
CD, UC (> 85 kg)	Four (4) vials (130 mg/26 mL) within the first eight (8) weeks of therapy	One (1) syringe (90 mg/mL) every eight (8) weeks

XXIV. Taltz (ixekizumab)

A. Approval Criteria

1. Psoriatic Arthritis (PsA)

When a benefit, coverage of Taltz may be approved when all of the following criteria are met (**a., b., and c.**):

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsA (see **Table 1**)

2. Plaque Psoriasis (PsO)

When a benefit, coverage of Taltz may be approved when all of the following criteria are met (**a. through d.**):

- a. The member is 6 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO
- c. The member meets one (1) of the following criteria (**i., ii., or iii.**):
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g., PUVA, UVB).

- ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
- iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**)

3. Ankylosing Spondylitis (AS)

When a benefit, coverage of Taltz may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of AS (see **Table 1**)

4. Non-radiographic Axial Spondyloarthritis (nr-axSpA)

When a benefit, coverage of Taltz may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of nr-axSpA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) nonsteroidal anti-inflammatory drugs (NSAIDs), or all NSAIDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of nr-axSpA (see **Table 1**)

B. Quantity Limitations

When prior authorization is approved, Taltz may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
Adult PsO with or without PsA	Eight (8) autoinjector/prefilled syringes (80 mg/mL) within the first twelve (12) weeks of therapy	One (1) autoinjector/prefilled syringe (80 mg/mL) every four (4) weeks
Pediatric PsO greater than 50 kg	Two (2) autoinjector/prefilled syringe (80 mg/mL) within the first 4 weeks of therapy	One (1) autoinjector/prefilled syringe (80 mg/mL) every four (4) weeks
Pediatric PsO 25- 50 kg	One (1) autoinjector/prefilled syringe (80 mg/mL) within the first 4 weeks of therapy	One (1) prefilled syringe (40 mg/0.5 mL) every four (4) weeks
Pediatric PsO less than 25 kg	One (1) prefilled syringe (40 mg/0.5 mL) within the first 4 weeks of therapy	One (1) prefilled syringe (20 mg/ 0.25 mL) every four (4) weeks
PsA, AS	Two (2) autoinjector/prefilled syringes (80 mg/mL) within the first four (4) weeks of therapy	One (1) autoinjector/prefilled syringe (80 mg/mL) every four (4) weeks
nr-axSpA	N/A	One (1) autoinjector/prefilled syringe (80 mg/mL) every four (4) weeks

N/A=Not Applicable

XXV. Tocilizumab (See Table 2 for Preferred Products)

A. Approval Criteria for Tocilizumab SC

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of tocilizumab SC may be approved when all of the following criteria are met **(a., through d.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe RA.
- c.** The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all nonbiologic DMARDs are contraindicated.
- d.** If the request is for a non-preferred tocilizumab SC product, the member has experienced therapeutic failure or intolerance to Actemra.

2. Giant Cell Arteritis (GCA)

When a benefit, coverage of tocilizumab SC may be approved when all of the following criteria are met **(a., through d.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of GCA.
- c.** The member has experienced therapeutic failure or intolerance to at least one (1) systemic corticosteroid (e.g., prednisone), or all corticosteroids are contraindicated.
- d.** If the request is for a non-preferred tocilizumab SC product, the member has experienced therapeutic failure or intolerance to Actemra.

3. Systemic Sclerosis-Associated Interstitial Lung Disease (SSc-ILD)

When a benefit, coverage of Actemra SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of SSc-ILD.
- c.** The member has experienced therapeutic failure or intolerance to at least one (1) immunosuppressant (e.g., mycophenolate mofetil, corticosteroids, cyclophosphamide) or all immunosuppressants are contraindicated.

4. Polyarticular Juvenile Idiopathic Arthritis (PJIA)

When a benefit, coverage of tocilizumab SC may be approved when all of the following criteria are met **(a., through d.)**:

- a.** The member is 2 years of age or older.
- b.** The member has a diagnosis of PJIA.
- c.** The member meets one (1) of the following criteria **(i. or ii.)**:
 - i.** The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
 - ii.** The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- d.** If the request is for a non-preferred tocilizumab SC product, the member has experienced therapeutic failure or intolerance to Actemra.

5. Systemic Juvenile Idiopathic Arthritis (SJIA)

When a benefit, coverage of tocilizumab SC may be approved when all of the following criteria are met **(a., b., and c.)**:

- a.** The member is 2 years of age or older.

- b. The member has a diagnosis of SJIA.
- c. If the request is for a non-preferred tocilizumab SC product, the member has experienced therapeutic failure or intolerance to Actemra.

B. Quantity Limitations

When prior authorization is approved, tocilizumab SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
RA and GCA	Four (4) prefilled syringes within the first four (4) weeks of therapy	Two (2) prefilled syringes every four (4) weeks -OR- Four (4) prefilled syringes every four (4) weeks
SSc-ILD	N/A	Four (4) prefilled syringes every four (4) weeks
PJIA	N/A	One (1) prefilled syringe every two (2) or three (3) weeks
SJIA	N/A	One (1) prefilled syringe every week or two (2) weeks

N/A=not applicable

C. Approval Criteria for Tocilizumab IV

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Tocilizumab IV may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe RA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) preferred intravenous biologic product for RA (i.e., infliximab, Simponi Aria).
- d. If the request is for Tofidence or Tyenne IV, the member has experienced therapeutic failure or intolerance to Actemra.

2. Giant Cell Arteritis (GCA)

When a benefit, coverage of Tocilizumab IV may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of GCA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) systemic corticosteroid (e.g., prednisone), or all corticosteroids are contraindicated.
- d. If the request is for Tofidence or Tyenne IV, the member has experienced therapeutic failure or intolerance to Actemra.

3. Polyarticular Juvenile Idiopathic Arthritis (PJIA)

When a benefit, coverage of Tocilizumab IV may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 2 years of age or older.
- b. The member has a diagnosis of PJIA.

- c. If the request is for Tofidence or Tyenne IV, the member has experienced therapeutic failure or intolerance to Actemra.
- 4. Systemic Juvenile Idiopathic Arthritis (SJIA)**
When a benefit, coverage of Tocilizumab IV may be approved when all of the following criteria are met **(a., b., and c.)**:
- a. The member is 2 years of age or older.
 - b. The member has a diagnosis of SJIA.
 - c. If the request is for Tofidence or Tyenne IV, the member has experienced therapeutic failure or intolerance to Actemra.
- 5. Chimeric antigen receptor (CAR) T cell-induced severe or life-threatening cytokine release syndrome (CRS)**
When a benefit, coverage of Actemra IV may be approved when all of the following criteria are met **(a. and b.)**:
- a. The member is 2 years of age or older.
 - b. The member has a diagnosis of CAR T cell-induced severe or life-threatening CRS.

XXVI. Tremfya (guselkumab)

A. Approval Criteria

1. Psoriatic Arthritis (PsA)

When a benefit, coverage of Tremfya may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsA (see **Table 1**)

2. Plaque Psoriasis (PsO)

When a benefit, coverage of Tremfya may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe PsO.
- c. The member meets one (1) of the following criteria **(i., ii., or iii.)**:
 - i. The member has experienced therapeutic failure or intolerance to phototherapy (e.g., PUVA, UVB).
 - ii. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., methotrexate).
 - iii. The member is contraindicated to both phototherapy and systemic therapy.
- d. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of PsO (see **Table 1**)

3. Ulcerative Colitis (UC)

When a benefit coverage of Tremfya may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).
- d. If the request is for Tremfya SC, the prescriber attests the member has already received three (3) induction doses of Tremfya IV within 3 to 4 months of initiating therapy with Tremfya SC or is currently undergoing induction therapy with Tremfya IV.

B. Quantity Limitations

When prior authorization is approved, Tremfya IV and Tremfya SC may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
PsA, PsO	Two (2) prefilled syringes/injectors (100 mg/mL) within the first four (4) weeks of therapy 28 days	One (1) prefilled syringe/injector (100 mg/mL) every eight (8) weeks
UC	Three (3) single-dose vials within the first twelve (12) to sixteen (16) weeks of therapy	One (1) prefilled syringe/injector (100 mg/mL) every eight (8) weeks -OR- One (1) prefilled syringe/pen (200 mg/2 mL) every four (4) weeks

XXVII. Velsipity (etrasimod)

A. Approval Criteria

1. Ulcerative Colitis (UC)

When a benefit, coverage of Velsipity may be approved when all of the following criteria are met **(a., b., and c.)**:

- The member has a diagnosis of moderately to severely active ulcerative colitis.
- The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., corticosteroids, azathioprine, cyclophosphamide), or all systemic therapies are contraindicated.
- The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).

XXVIII. Xeljanz (tofacitinib)

A. Approval Criteria

1. Rheumatoid Arthritis (RA)

When a benefit, coverage of Xeljanz or Xeljanz XR may be approved when all of the following criteria are met **(a. through d.)**:

- The member is 18 years of age or older.
- The member has a diagnosis of moderate to severely active RA.
- The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

2. Juvenile Idiopathic Arthritis (JIA)

When a benefit, coverage of Xeljanz tablet or Xeljanz oral solution may be approved when all of the following criteria are met **(a. through d.)**:

- The member is 2 years of age or older.
- The member has a diagnosis of JIA.
- The member meets one (1) of the following criteria **(i. or ii.)**:

- i. The member has experienced therapeutic failure or intolerance to at least one (1) non-biologic DMARD (e.g., methotrexate, leflunomide, sulfasalazine, cyclosporine), or all non-biologic DMARDs are contraindicated.
- ii. The member requires initial biologic therapy due to involvement of high-risk joints (e.g., cervical spine, wrist, or hip), high disease activity, and/or those judged by their physician to be at high risk of disabling joint damage.
- d. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

3. Psoriatic Arthritis (PsA)

When a benefit, coverage of Xeljanz or Xeljanz XR may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of PsA.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

4. Ulcerative Colitis (UC)

When a benefit, coverage of Xeljanz or Xeljanz XR may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of moderate to severe UC.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

6. Ankylosing Spondylitis (AS)

When a benefit, coverage of Xeljanz or Xeljanz XR may be approved when all of the following criteria are met **(a. through d.)**:

- a. The member is 18 years of age or older.
- b. The member has a diagnosis of AS.
- c. The member has experienced therapeutic failure or intolerance to at least one (1) nonsteroidal anti-inflammatory drug (NSAID), or all NSAIDs are contraindicated.
- d. The member has experienced therapeutic failure or intolerance to at least one (1) tumor necrosis factor (TNF) blocker therapy.

XXIX. Zeposia (ozanimod)

A. Approval Criteria

1. Multiple Sclerosis (MS)

When a benefit, coverage of Zeposia may be approved when the following criterion is met **(a.)**:

- a. The member has a diagnosis of a relapsing form of multiple sclerosis (e.g., clinically isolated syndrome, relapsing-remitting disease, or active secondary progressive disease).

2. Ulcerative Colitis (UC)

When a benefit, coverage of Zeposia may be approved when all of the following criteria are met **(a., b., and c.)**:

- a. The member has a diagnosis of moderately to severely active ulcerative colitis.
- b. The member has experienced therapeutic failure or intolerance to at least one (1) systemic therapy (e.g., corticosteroids, azathioprine, cyclophosphamide), or all systemic therapies are contraindicated.
- c. The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).

XXXI. Zymfentra (infliximab-dyyb) SC

A. Approval Criteria

1. Ulcerative Colitis (UC)

When a benefit, coverage of Zymfentra may be approved when all of the following criteria are met **(a. through d.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe UC.
- c.** The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of UC (see **Table 1**).
- d.** The prescriber attests the member has already received at least three doses of infliximab IV at least 10 weeks before initiating therapy with Zymfentra or is currently undergoing induction therapy with infliximab IV.

2. Crohn's Disease (CD)

When a benefit, coverage of Zymfentra may be approved when all of the following criteria are met **(a. through d.)**:

- a.** The member is 18 years of age or older.
- b.** The member has a diagnosis of moderate to severe CD.
- c.** The member has experienced therapeutic failure or intolerance to at least two (2) preferred biologic products for the treatment of CD (see **Table 1**).
- d.** The prescriber attests the member has already received at least three doses of infliximab IV at least 10 weeks before initiating therapy with Zymfentra or is currently undergoing induction therapy with infliximab IV.

B. Quantity Limitations

When prior authorization is approved, Zymfentra may be authorized in quantities as follows:

Diagnosis	Induction Therapy	Maintenance Therapy
UC, CD	N/A	Two (2) prefilled pens/syringes every four (4) weeks

XXXII. If the patient has already had a trial of at least one (1) biologic agent the patient is not required to “step back” and try a nonbiologic agent.

XXXIII. For Medicare Part D beneficiaries, drugs(s) addressed in this policy may be approved when used for a medically accepted indication as defined by the Centers for Medicare & Medicaid Services (CMS).

Limitations of Coverage

- I.** Olumiant, Actemra IV and Tofidence for treatment of COVID-19 in hospitalized patients are not eligible for Medicare Part D coverage as drugs administered in an inpatient setting are covered under Medicare Part A.
- II.** Combination use of disease modifying MS agents with Zeposia (e.g., Aubagio, Gilenya, interferons, Copaxone, Tysabri, etc.) will not be authorized.

Authorization Duration

- Medicare Part D Plans:
 - **Multiple Sclerosis Only (Zeposia):** If approved, a 24 month authorization will be granted.
 - **All Other Indications:** If approved, a 12 month authorization will be granted.
 - Note: For induction therapy authorization duration, refer to the Quantity Limitations tables for the respective drug and diagnosis.

Automatic Approval Criteria

None

For previous versions of the Zeposia Medicare policy, please see policy [J-0296](#).

References:

1. Actemra [package insert]. San Francisco, CA: Genentech; March 2021.
2. Cimzia [package insert]. Smyrna, GA: UCB, Inc.; December 2022.
3. Cosentyx [package insert]. East Hanover, NJ: Novartis; October 2023.
4. Enbrel [package insert]. Thousand Oaks, CA: Immunex Corporation; October 2023.
5. Humira [package insert]. North Chicago, IL: AbbVie, Inc.; February 2021.
6. Ilumya [package insert]. Cranbury, NJ: Sun Pharmaceutical Industries, Inc.; December 2022.
7. Kevzara [package insert]. Bridgewater, NJ and Tarrytown, NY: Sanofi-Aventis and Regeneron; June 2024.
8. Kineret [package insert]. Waltham, MA: SOBI, Inc. (Swedish Orphan Biovitrum); December 2020.
9. Olumiant [package insert]. Indianapolis, IN: Eli Lilly; December 2021.
10. Ocrencia [package insert]. Princeton, NJ: Bristol-Myers Squibb; October 2023.
11. Otezla [package insert]. Thousand Oaks, CA: Amgen Inc.; April 2024.
12. Remicade [package insert]. Horsham, PA: Janssen Biotech, Inc.; October 2021.
13. Rinvoq [package insert] North Chicago, IL: AbbVie, Inc.; April 2024.
14. Siliq [package insert]. Bridgewater, NJ: Valeant Pharmaceuticals International; February 2017.
15. Simponi [package insert]. Horsham, PA: Janssen Biotech, Inc.; March 2023.
16. Simponi Aria [package insert]. Horsham, PA: Janssen Biotech, Inc.; March 2023.
17. Skyrizi [package insert]. North Chicago, IL: AbbVie, Inc.; June 2024.
18. Stelara [package insert]. Horsham, PA: Janssen Biotech, Inc.; March 2023.
19. Taltz [package insert]. Indianapolis, IN: Eli Lilly and Company; February 2024.
20. Tremfya [package insert]. Horsham, PA: Janssen Biotech, Inc.; September 2024.
21. Xeljanz, Xeljanz XR [package insert]. New York, NY: Pfizer, Inc.; December 2021.
22. Langley R., Elewski B, Lebwohl M et al. Secukinumab in plaque Psoriasis. Results of Two Phase 3 trials. *N Engl J Med*. 2014 July 24; 371:326-338.
23. Takei S, Groh D, Bernstein B, et al: Safety and efficacy of high dose etanercept in treatment of juvenile rheumatoid arthritis. *J Rheumatol* 2001; 28(7):1677-1680.
24. Leonardi CL, Powers JL, Matheson RT, et al: Etanercept as monotherapy in patients with psoriasis. *N Engl J Med* 2003; 349(21):2014-2022.
25. Gorman JD, Sack KE, & Davis JC Jr: Treatment of ankylosing spondylitis by inhibition of tumor necrosis factor alpha. *N Engl J Med* 2002; 346(18):1349-1356.
26. Leonardi CL, Powers JL, Matheson RT, et al. Etanercept as Monotherapy in Patients with Psoriasis. *N Engl J Med*. 2003;349: 2014-2022.
27. Colombel JF, Sandborn WJ, Rutgeerts P, et al: Adalimumab for maintenance of clinical response and remission in patients with Crohn's disease: the CHARM trial. *Gastroenterology* 2007; 132(1):52-65.
28. Gladman DD, Mease PJ, Cifaldi MA, et al: Adalimumab improves joint-related and skin-related functional impairment in patients with psoriatic arthritis: patient-reported outcomes of the Adalimumab Effectiveness in Psoriatic Arthritis Trial. *Ann Rheum Dis* 2007; 66(2):163-168.

29. Gordon KB, Langley RG, Leonardi C, et al: Clinical response to adalimumab treatment in patients with moderate to severe psoriasis: double-blind, randomized controlled trial and open-label extension study. *J Am Acad Dermatol* 2006; 55(4):598-606.
30. Mease PJ, Gladman DD, Ritchlin CT, et al: Adalimumab for the treatment of patients with moderately to severely active psoriatic arthritis: results of a double-blind, randomized, placebo-controlled trial. *Arthritis Rheum* 2005; 52(10):3279-3289.
31. Menter A, Tying SK, Gordon K, et al: Adalimumab therapy for moderate to severe psoriasis: a randomized, controlled phase III trial. *J Am Acad Dermatol* 2008; 58(1):106-115.
32. Peters BP, Weissman FG, Gill MA. Pathophysiology and treatment of psoriasis. *Am J Health Sys Pharm.* 2000; 57: 645-662.
33. Griffiths CE, Strober BE, van de Kerkhof P, et al: Comparison of ustekinumab and etanercept for moderate-to-severe psoriasis. *N Engl J Med* 2010; 362(2):118-128.
34. Leonardi CL, Kimball AB, Papp KA, et al: Efficacy and safety of ustekinumab, a human interleukin-12/23 monoclonal antibody, in patients with psoriasis: 76-week results from a randomised, double-blind, placebo-controlled trial (PHOENIX 1). *Lancet* 2008; 371(9625):1665-1674.
35. Papp KA, Langley RG, Lebwohl M, et al: Efficacy and safety of ustekinumab, a human interleukin-12/23 monoclonal antibody, in patients with psoriasis: 52-week results from a randomised, double-blind, placebo-controlled trial (PHOENIX 2). *Lancet* 2008; 371(9625):1675-1684.
36. Meyer DM, Jesson MI, Li X, et al. Anti-inflammatory activity and neutrophil reductions mediated by the JAK1/JAK3 inhibitor, CP-690,550, in rat adjuvant-induced arthritis. *J Inflamm (Lond).* 2010; 7:41.
37. Sandborn W., Feagan B., Stoinov S. et al. Certolizumab pegol for the treatment of Crohn's disease. *N Eng J Med* 2007; 357:228-38.
38. Schreiber S., Kareemi M., Lawrence I. et al. Maintenance therapy with certolizumab pegol for Crohn's disease. *N Eng J Med* 2007; 357:239-50.
39. Genovese MC, Fleischmann R, Kivitz AJ, et al. Sarilumab plus methotrexate in patients with active rheumatoid arthritis and inadequate response to methotrexate: results of a phase III study (MOBILITY). *Arthritis Rheumatol.* 2015 Jun;67(6):1424-37.
40. Fleischmann R, van Adelsberg J, Lin Y, et al. Sarilumab and nonbiologic disease-modifying antirheumatic drugs in patients with active rheumatoid arthritis and inadequate response or intolerance to tumor necrosis factor inhibitors (TARGET). *Arthritis Rheumatol.* 2017 Feb;69(2):277-290.
41. Jiang Y, Genant HK, Watt I, et al: A multicenter, double-blind, dose-ranging, randomized, placebo-controlled study of recombinant human interleukin-1 receptor antagonist in patients with rheumatoid arthritis. Radiologic progression and correlation of Genant and Larsen scores. *Arthritis Rheum* 2000; 43(5):1001-1009.
42. Jiang Y, Genant HK, Watt I, et al: A multicenter, double-blind, dose-ranging, randomized, placebo-controlled study of recombinant human interleukin-1 receptor antagonist in patients with rheumatoid arthritis. Radiologic progression and correlation of Genant and Larsen scores. *Arthritis Rheum* 2000a; 43(5):1001-1009.
43. Lebwohl M, Strober B, Menter A, et al. Phase 3 Studies Comparing Brodalumab with Ustekinumab in Psoriasis. *NEJM* 373(Oct). 2015:1318-1328.
44. Zhou H, Jang H, Fleischmann RM, et al. Pharmacokinetics and safety of golimumab, a fully human anti-TNF-alpha monoclonal antibody, in subjects with rheumatoid arthritis. *J Clin Pharmacol.* 2007;47(3):383-396.
45. Griffiths CEM, Reich K, Lebwohl M, et al. Comparison of ixekizumab with etanercept or placebo in moderate-to-severe psoriasis (UNCOVER-2 and UNCOVER-3): results from two phase 3 randomised trials. *Lancet* 2015; 386: 541-51.
46. Singh, Jasvinder A, Saag KG, Bridges SL, et al. 2015 American College of Rheumatology Guideline for the Treatment of Rheumatoid Arthritis. *Arthritis & Rheumatol* Jan (68) (2016):1-26.
47. Braun, J. von, et al. "2010 update of the ASAS/EULAR recommendations for the management of ankylosing spondylitis." *Annals of the rheumatic diseases* 70.6 (2011): 896-904.
48. Lichtenstein GR, Loftus EV, Isaacs KL, et al. The American College of Gastroenterology Guidelines - Management of Crohn's Disease in Adults. *Am J Gastroenterol.* 2018; 113: 481-517.
49. Menter A, Gottlieb A, Feldman SR, Van Voorhees AS, Leonardi CL, Gordon KB, et al. Guidelines of care for the management of psoriasis and psoriatic arthritis: Section 1. Overview of psoriasis and

- guidelines of care for the treatment of psoriasis with biologics. *Journal of the American Academy of Dermatology* 58.5 (2008):826-50.
50. Gottlieb, Alice, et al. Guidelines of care for the management of psoriasis and psoriatic arthritis: Section 2. Psoriatic arthritis: overview and guidelines of care for treatment with an emphasis on the biologics. *Journal of the American Academy of Dermatology* 58.5 (2008): 851-864.
51. Ringold S, Weiss PF, Colbert RA et al. Childhood Arthritis and Rheumatology Research Alliance Consensus Treatment plans New Onset Polyarticular Juvenile Idiopathic Arthritis. *Arthritis Care Res (Hoboken)*. 2014. July; 66(7):1063-72.
52. Mahadevan Uma, et al. Inflammatory Bowel Diseases in Pregnancy Clinical Care Pathway: A Report from the American Gastroenterological Association IBD Parenthood Project Working Group. *Gastroenterology*. Available at: [https://www.gastrojournal.org/article/S0016-5085\(18\)35437-4/fulltext](https://www.gastrojournal.org/article/S0016-5085(18)35437-4/fulltext). Accessed March 1, 2019.
53. Menter A, Strober BE, Kaplan DH, et al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis with biologics. *J Am Acad Dermatol*. 2019;80:1029-72.
54. Singh JA, Guyatt G, Ogdie A, et al. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. *Arthritis Care Res*. 2019 Jan;71(1):2-29.
55. Ringold S, Angeles-Han ST, Beukelman T, et al. 2019 American College of Rheumatology/Arthritis Foundation Guideline for the Treatment of Juvenile Idiopathic Arthritis: Therapeutic Approaches for Non-Systemic Polyarthritis, Sacroiliitis, and Enthesitis. *Arthritis Rheumatol*. 2019 Jun;71(6):846-863.
56. Rubin DT, Ananthakrishnan AN, Siegel CA, et al. ACG Clinical Guideline: Ulcerative Colitis in Adults. *Am J Gastroenterol*. 2019 Mar;114(3):384-413.
57. Hatemi G, Christensen R, Bang D, et al. 2018 update of the EULAR recommendations for the management of Behçet's syndrome. *Ann Rheum Dis*. 2018 Jun;77(6):808-818.
58. Feuerstein JD, Ho EY, Shmidt E, et al. AGA Clinical Practice Guidelines on the Medical Management of Moderate to Severe Luminal and Perianal Fistulizing Crohn's Disease. *Gastroenterology*. 2021;160(7):2496-2508.
59. Rathod DG. Phototherapy. StatPearls. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK563140/>. Accessed September 3, 2021.
60. Center for Drug Evaluation and Research. Serious heart events, cancer, blood clots for certain jak inhibitors. U.S. Food and Drug Administration. Available at: <https://www.fda.gov/drugs/drug-safety-and-availability/fda-requires-warnings-about-increased-risk-serious-heart-related-events-cancer-blood-clots-and-death>. Accessed September 15, 2021.
61. Center for Drug Evaluation and Research. Serious heart events, cancer, blood clots for certain jak inhibitors. U.S. Food and Drug Administration. Available at: <https://www.fda.gov/drugs/drug-safety-and-availability/fda-requires-warnings-about-increased-risk-serious-heart-related-events-cancer-blood-clots-and-death>. Accessed September 15, 2021.
62. Elston DM. American Academy of Dermatology and national psoriasis foundation guidelines of care for the management and treatment of psoriasis. *Journal of the American Academy of Dermatology*. 2021;84(2):257-258.
63. Approved risk evaluation and mitigation strategies (REMS). [accessdata.fda.gov](https://www.accessdata.fda.gov/scripts/cder/remis/index.cfm?event=IndvRemsDetails.page&REMS=362). Available at: <https://www.accessdata.fda.gov/scripts/cder/remis/index.cfm?event=IndvRemsDetails.page&REMS=362>. Accessed November 15, 2021.
64. Sotyktu [package insert]. Princeton, NJ: Bristol-Myers Squibb Company; September 2022.
65. Amjevita [package insert]. Thousand Oaks, CA: Amgen Inc.; August 2023.
66. Zeposia [package insert]. Princeton, NJ: Bristol-Myers Squibb Company; August 2023.
67. National Multiple Sclerosis Society. Types of MS. Available at: <https://www.nationalmssociety.org/What-is-MS/Types-of-MS>. Accessed May 3, 2022.
68. Litfulo [package insert]. New York, NY: Pfizer Inc.; June 2023.
69. Hadlima [package insert]. Jersey City, NJ: Organon & Co.; July 2023.
70. Hyrimoz [package insert]. Princeton, NJ: Sandoz; September 2023.
71. Cyltezo [package insert]. Ridgefield, CT: Boehringer Ingelheim Pharmaceuticals, Inc.; July 2023.
72. Hulio [package insert]. Morgantown, WV: Mylan Pharmaceuticals Inc.; August 2023.
73. Yusimry [package insert]. Redwood City, CA: Coherus BioSciences, Inc.; September 2023.

74. Idacio [package insert]. Lake Zurich, IL: Fresenius Kabi USA; October 2023.
75. Yuflyma [package insert]. Jersey City, NJ: Celltrion USA, Inc.; October 2023.
76. Abrilada [package insert]. New York, NY: Pfizer Inc.; October 2023.
77. Velsipity [package insert]. New York, NY: Pfizer Inc.; October 2023.
78. Bimzelx [package insert]. Smyrna, GA: UCB, Inc.; October 2023.
79. Entyvio [package insert]. Lexington, MA: Takeda Pharmaceuticals U.S.A., Inc.; April 2024.
80. Omvoh [package insert]. Indianapolis, IN: Eli Lilly and Company; October 2023.
81. Zymfentra [package insert]. Jersey City, NJ: CELLTRION USA, Inc.; October 2023.
82. Simlandi [package insert]. Leesburg, VA: Alvotek USA Inc.; February 2024.
83. Tyenne [package insert]. Lake Zurich, IL: Fresenius Kabi USA LLC; March 2024.
84. Tofidence [package insert]. Cambridge, MA: Biogen MA Inc.; July 2024.
85. Fraenkel L, Bathon JM, England BR, et al. American College of Rheumatology guideline for the treatment of rheumatoid arthritis. *Arthritis Care & Research*. 2021;73(7):924-939
86. Onel KB, et al. 2021 American College of Rheumatology Guideline for the Treatment of Juvenile Idiopathic Arthritis: Therapeutic Approaches for Oligoarthritis, Temporomandibular Joint Arthritis, and Systemic Juvenile Idiopathic Arthritis. *Arthritis Care Res (Hoboken)*. 2022 Apr;74(4):521-537.
87. Onel KB, Horton DB, Lovell DJ, et al. 2021 American College of Rheumatology guideline for the treatment of juvenile idiopathic arthritis: therapeutic approaches for oligoarthritis, temporomandibular joint arthritis, and systemic juvenile idiopathic arthritis. *Arthritis & Rheumatology*. 2022; 74(4): 553-569.
88. Maz M, Chung SA, Abril A, et al. 2021 American College of Rheumatology/Vasculitis Foundation guideline for the management of giant cell arteritis and Takayasu arteritis. *Arthritis Care Res*. 2021; 73(8): 1071-1087.

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